

Secondary Amenorrhoea/Oligomenorrhoea Pathway

This pathway has been developed from published guidance, in collaboration with local gynaecologists.

This guidance is to assist GPs in decision making and is not intended to replace clinical judgment.

Primary amenorrhoea (not included in this pathway): by 14 if no secondary sexual characteristics or by 16 with normal sexual characteristics
Secondary amenorrhoea: no menstrual bleeding for 6 months.
Oligomenorrhoea: periods that occur at intervals of 35 days to 6 months with less than 9 cycles per year.

Pathway under review

History:

- Age menarche and menstrual history
- Recent weight change, dieting (refer to eating disorders if applicable)
- Exercise history/ stress/ travel
- Medical drug history – prescribed and illicit drugs
- Sexual history and contraception
- Obstetric history
- NB delay in investigation until 9 months if obvious cause e.g. exercise – sports person or ballet dancer

Examination:

- Height, weight and BMI
- Hirsutism
- Skin (acne)
- Visual fields

Investigations:

- Exclude pregnancy
- Thyroid function tests
- Hormone profile: Day 2 – 7 (or randomly if no period) FSH/LH/ oestradiol/prolactin/testosterone
- Fasting glucose / glycosylated haemoglobin if obese
- Pelvic scan to assess endometrial thickness and look for polycystic ovaries.
- Bone density scan in women with low oestrogen and amenorrhoea

Patient trying to conceive

[Infertility Pathway](#)

Normal Investigations

Reassure, advise regarding contraception. Review 6 months. If persists, repeat all tests

[Refer to Gynaecology](#)

Polycystic Ovaries

[POCS Pathway](#)
Advise re: endometrial protection

Advise re weight loss if raised BMI

Do not initiate insulin sensitising medications eg. Metformin in primary care
Offer weight loss advice and management. Treat hirsutism and acne.

Raised Androgen

[Refer to Endocrinology](#)

Raised Prolactin

Consider causative medications e.g. risperidone, olanzapine, SSRIs, phenothiazines, metoclopramide. Do not stop mental health medication without specialist advice

Prolactin > 1000mu/L
Repeat in 6 weeks
Check macroprolactin level

Prolactin 400 – 1000mu/L

Give combined [oral contraceptive pill](#)
Repeat test in 3 months

If still 400-1000mu/L continue COC

Prolactin > 1000 mu/L
?prolactinoma

[Refer to Endocrinology](#)

Hypogonadotrophic hypogonadism

Low oestrogen and low FSH

Offer weight gain advice +/- dietician referral if appropriate. Refer to mental health if eating disorder identified

If prolonged amenorrhoea, check bone density and consider oestrogen supplements

Menopause/ premature ovarian insufficiency

Raised FSH/LH (consider repeating in 6 weeks if <45 years)

[Management of Menopause guidance](#)

Links:

[Patient information leaflet Absent or Irregular Periods](#)

Pathway created by NCL led by Camden CCG Clinical Cabinet + GB July 2016

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Refer to current BNF or SPC for full medicines information

Review due – March 2020

This pathway is currently under review – as some of the content may be out of date, it should be viewed as an archive document for information only. Please contact Camden MMT for up-to-date information/advice.